A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

22 April 2026

Dr Franziska Levin
604-610 Glenhuntly Road

Dear Dr Levin,

Re: Mr Andreas Makris
DOB: 1946-02-28
Address: 5 Brockenshire Street, CLIFTON HILL, VIC, 3068

Summary

- **Liver cirrhosis remains stable with no evidence of hepatocellular carcinoma on surveillance imaging.**
- **CT scan findings reviewed in the context of a potential hernia repair by Mr Benjamin Thomson**
- **Encouraged increased protein intake to mitigate risks of sarcopenia.**
- **For six-monthly surveillance with ultrasound and blood tests.**
- **No action required by you.**

I had the pleasure of conducting a review with Andreas Makris. You will be pleased to know that his recent investigations, including a CT scan and an ultrasound, remain stable when compared to imaging from six months ago. There is no evidence of new pathology or suspicious lesions concerning for hepatocellular carcinoma.

Andreas continues to manage his cirrhosis well. He has achieved a modest weight loss of approximately 1.5 kilograms through lifestyle modifications. We discussed his recent CT scan, which was requested by his surgeon, Mr Benjamin, as part of the preoperative planning for a hernia repair. The scan was specifically utilised to evaluate the presence of dilated collateral veins and portal hypertension, which would increase the risk of intraoperative haemorrhage. Currently, the plan remains conservative, and surgery will only be pursued if his symptoms escalate.

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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His diet is generally excellent, consisting primarily of fresh produce. However, I have counselled him on the importance of maintaining a high-protein diet. In the context of cirrhosis, adequate protein intake is vital to prevent muscle wasting and sarcopenia, which are significant prognostic factors. We also discussed his exercise regimen; he continues to participate in Pilates and regular walking.

Impression and Plan

Andreas's condition is stable, and we will continue with his current management strategy. From a gastroenterology perspective, I have organised a repeat abdominal ultrasound and blood tests to be performed in six months for ongoing surveillance. I have provided him with the necessary pathology and radiology referrals via his mobile phone. I will review him again once those results are available.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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